

Student Safety & Behavior Risk Screener

A School-Administered Screening Instrument for Bullying, Suicidal Thoughts & Tendencies, Physical Aggression, Violence Attitudes, Stress & Pressure, and Media Exposure — Ages 6–50

For use by trained school counselors, psychologists, or other designated mental health professionals only. This is an original screening instrument, not a diagnostic tool, and does not replace clinical judgment. Read the Critical Safety Notice (next page) and the Administration Guidelines before use.

Version 1.2 · 2026

61 items across 5 scored screening domains, plus a dedicated suicide-risk decision protocol and a descriptive media-exposure profile

READ BEFORE ADMINISTERING

This instrument screens for suicidal thoughts and tendencies (Section 3). Screening for suicide risk carries real responsibility. Follow these rules without exception:

- Only trained, school-designated staff should administer this screener. Only a licensed or credentialed mental health professional may interpret suicide-risk answers and decide on risk level^[11].
- Any positive answer in Section 3 means STOP: move immediately to the Immediate Response Protocol (page 23). Do not wait until the full questionnaire is finished.
- Never leave a student alone after a positive suicide-risk screen -- not even briefly.
- This is a screener, not a diagnosis. A positive result means “refer now for a full evaluation,” not “this student is suicidal.” Asking these questions directly does not plant the idea or increase risk^[6].
- Before you begin any screening session, have your school's crisis/referral protocol, parent/guardian contact information, and the crisis resources on page 24 ready.

If a student is in immediate danger right now: treat it as a medical emergency. In India, call Tele-MANAS at 14416 or your local emergency number; in the United States, call or text 988. Do not wait for this document's full protocol in a life-threatening moment.

About This Instrument

The Student Safety & Behavior Risk Screener (SSBRS) is an original instrument designed to help trained school personnel identify students who may need closer support or referral across five related risk domains, plus a descriptive media-exposure profile that adds context without contributing to any score. It was designed by reviewing publicly documented, peer-reviewed psychological screening literature -- it does not reproduce copyrighted item text from any existing published instrument, with the exception of the public-domain DASS-21 stress items noted below.

What each domain draws on

Bullying (Sections 1-2). The repetition-plus-power-imbalance definition of bullying, and the decision to score being-bullied and bullying-others as separate subscales, follows the Olweus Bully/Victim Questionnaire framework^[1] and the Illinois Bully Scale's separation of bullying, fighting, and victimization^[2]. The self-defense/power-imbalance item (item 8) is inspired by the California Bully Victimization Scale-Retrospective's approach to measuring power imbalance directly^[3].

Suicidal thoughts & tendencies (Section 3). Item wording and the decision logic closely follow the NIMH Ask Suicide-Screening Questions (ASQ) toolkit^[4] and the Columbia-Suicide Severity Rating Scale (C-SSRS) screener^[5]. Both are explicitly published by their developers for free reuse in clinical and educational settings with no licensing fee^[5], so this section uses their validated structure directly rather than rewording safety-critical questions.

Physical aggression (Section 4). The reactive/proactive split follows Raine and colleagues' Reactive-Proactive Aggression Questionnaire framework^[6], and the physical/verbal/anger dimensions draw on Buss & Perry's four-factor aggression model^[7].

Violence attitudes (Section 5). Items measuring enjoyment of and endorsement of violence draw on the Normative Beliefs About Aggression Scale^[8] and CDC-compiled attitude-toward-violence measures^[9], adapted into original wording appropriate for a broad age range.

Stress & pressure (Section 6). The academic-stress items are original, written to reflect the workload, exam-pressure, grade-worry, and parental/teacher-expectation domains documented in validated academic stress scales such as the Educational Stress Scale for Adolescents^[18] and the India-validated Adolescence Stress Scale^[19]. The general life-stress items use the Stress subscale of the Depression Anxiety Stress Scales (DASS-21)^[20], which its developers have explicitly placed in the public domain for free reuse^[20], so this section reproduces its validated wording and severity bands directly^{[21][22]}.

Media exposure (Section 7, descriptive only). The screen-time bands follow the format used in the HBSC international screen-time survey protocol^[26]. The content-type checklists and violent-content frequency items are original, written to capture the same underlying construct as the Exposure to Violent Media Questionnaire^[24] and the Content-based Media Exposure Scale (C-ME)^[23], which both pair how much media a person consumes with how violent that content is. Unlike those instruments, this section is not converted into a numeric exposure score -- it stays descriptive, consistent with the user's request to capture patterns rather than compute a band.

Section	Domain	Items	What it flags
---------	--------	-------	---------------

1	Bullying -- Victimization	8	Being bullied by peers
2	Bullying -- Perpetration	8	Bullying others
3	Suicidal Thoughts & Tendencies	6	Suicide risk -- decision tree, not summed
4	Physical Aggression	12	Reactive & proactive aggressive behavior
5	Violence Attitudes	10	Endorsement / enjoyment of violence
6	Stress & Pressure	14	Academic and general life stress levels
7	Media Exposure	9	Screen time & content patterns -- descriptive only, not summed

61 items (52 scored across five domains, plus 9 descriptive media-exposure items) plus the 6-question suicide-risk protocol. Full administration for one student typically takes 25-35 minutes, depending on age and how many follow-up questions are triggered.

Administration Guidelines

Before you screen anyone

- Put your school's response system in place first: a designated on-site mental health professional, a referral network, and a written crisis protocol, before you start screening students^[11].
- Train every staff member who will administer this screener, even briefly, in what to do with a positive answer -- the Immediate Response Protocol on page 23 should be second nature to them^[11].
- Confirm you have capacity to follow up: screening reliably increases the number of students who need a mental health referral^[11].

Consent & confidentiality

Given the sensitivity of the suicide-risk content, obtain active, informed written consent from a parent or guardian before screening a minor, and explain the purpose of the screener in age-appropriate terms to the student^[11]. Keep every result confidential, share it only with those directly responsible for the student's safety, and document each step^[11]. Consult your school's child-protection and data-privacy policies -- and any applicable national or state regulations -- before finalizing your consent and confidentiality procedures, since these vary by jurisdiction and this document does not constitute legal advice.

Who may administer vs. who may interpret

Any trained staff member may **administer** Sections 1, 2, 4, 5, 6, and 7. Section 3 (suicide risk) may be given by trained staff as a first-pass screen^[5], but only a licensed or credentialed mental health professional may **interpret** the results and decide on a risk level or course of action^[11].

Setting

Administer one-on-one, in a private and quiet space, in a calm and non-judgmental tone, with no time pressure. Read items aloud for younger children or students with lower reading levels.

Age-adaptation guide

Age band	Adaptation
Ages 6-9	Read every item aloud, one at a time. Use the 3-point Simple Scale (Never / Sometimes / A lot) instead of the 5-point scale. Pair Section 3 with parent/teacher behavioral observation -- validated self-report suicide screening is not well established below age 8 ^[9] , and should only proceed under direct guidance of a mental health professional.
Ages 10-17	Standard self-report with the 5-point frequency scale (Sections 1-2) and the agree/disagree scale (Section 5). Section 3 may be self-administered using the printed wording, which is validated from age 8 upward ^[4] .
Ages 18-50	Standard self-report as printed. For adult or staff populations, Sections 1-2 may be reframed retrospectively ("when you were in school..."), an approach modeled on the California Bully Victimization Scale-Retrospective ^[3] .
Section 7 note (all ages)	For ages 6-9, complete the screen-time items with caregiver assistance or via caregiver report -- young children's self-reported hours per day are unreliable ^[26] . For all ages, Section 7 stays descriptive and is never converted into a score.

Rating Scales Used in This Instrument

Seven different response scales are used across the seven sections. Introduce the relevant scale before each section and make sure the respondent understands it before beginning.

Standard Frequency Scale (Sections 1-2 & 6-academic, ages 10+)

"In the past 30 days, how often has this happened?"

0	1	2	3	4
Never	Once or twice	2-3x a month	About 1x a week	Several x a week+

Simple Scale (Sections 1-2, ages 6-9)

Same items, simplified for younger children.

0	1	2
Never	Sometimes	A lot

Aggression Scale (Section 4)

"How often does this describe you?"

0	1	2
Never	Sometimes	Often

Agreement Scale (Section 5)

"How much do you agree with this statement?"

1	2	3	4	5
Strongly disagree	Disagree	Neutral	Agree	Strongly agree

Stress Scale (Section 6 -- general life stress, ages 12+)

"Over the past week, how much did this apply to you?"

0	1	2	3
Did not apply to me at all	Applied to me to some degree, or some of the time	Applied to me to a considerable degree, or a good part of time	Applied to me very much, or most of the time

Screen Time Scale (Section 7, descriptive only)

"On an average day, about how much time do you spend on this?"

0	1	2	3	4
Less than 1 hour	1-2 hours	2-4 hours	4-6 hours	More than 6 hours

Media Frequency Scale (Section 7, descriptive only)

"How often does this happen when you use social media or watch TV/movies?"

0	1	2	3	4
Never	Rarely	Sometimes	Often	Very often

Section 3 (suicidal thoughts & tendencies) uses **Yes / No** answers with branching follow-up questions, not a frequency scale -- see page 10^[4]. Section 7 also includes two Yes/No-style items and two multi-select checklists; none of these are summed or converted into a band.

SECTION 1 — BULLYING: BEING BULLIED

In the past 30 days, how often has this happened to you?

Bullying is behavior that is **repeated**, **intended to hurt**, and involves an **imbalance of power** (physical, social, or numerical) between the people involved. It is different from a single argument or a fight between people of equal standing.

1. Other students called me hurtful names, made fun of me, or teased me in a way that upset me.	0 1 2 3 4
2. Other students left me out on purpose, or told others not to be friends with me.	0 1 2 3 4
3. Other students spread rumors or untrue stories about me.	0 1 2 3 4
4. Other students hit, kicked, pushed, or shoved me in a way that hurt.	0 1 2 3 4
5. Other students took or damaged my belongings on purpose.	0 1 2 3 4
6. Other students threatened to hurt me or someone I care about.	0 1 2 3 4
7. Other students sent me mean messages, posted embarrassing things about me, or left me out online.	0 1 2 3 4
8. When these things happened, it was hard for me to make them stop or defend myself.	0 1 2 3 4

Scale: 0 = Never · 1 = Once or twice · 2 = 2-3x a month · 3 = About once a week · 4 = Several times a week or more

SECTION 2 — BULLYING: BULLYING OTHERS

In the past 30 days, how often have you done this?

These questions ask about your own behavior toward other people over the same time period. Answer honestly -- there is no penalty for honest answers, and honest answers help identify the right kind of support.

9. I called another student hurtful names, made fun of them, or teased them in a way I knew would upset them.	0 1 2 3 4
10. I left another student out on purpose, or told others not to be friends with them.	0 1 2 3 4
11. I spread rumors or untrue stories about another student.	0 1 2 3 4
12. I hit, kicked, pushed, or shoved another student in a way that could hurt them.	0 1 2 3 4
13. I took or damaged another student's belongings on purpose.	0 1 2 3 4
14. I threatened to hurt another student or someone they care about.	0 1 2 3 4
15. I sent mean messages about another student, posted embarrassing things about them, or got others to leave them out online.	0 1 2 3 4
16. I kept doing these things even after I could tell the other person wanted it to stop.	0 1 2 3 4

Scale: 0 = Never · 1 = Once or twice · 2 = 2-3x a month · 3 = About once a week · 4 = Several times a week or more

Section 3 — Suicidal Thoughts & Tendencies

Ask in private, one-on-one, in a calm tone. Read each question exactly as written^[4]. If the respondent is under age 8, use behavioral observation and parent/teacher report instead of self-report^[9].

This section is not scored by adding up points. It works as a decision tree: any “yes” triggers the next question, and the pattern of answers determines the risk category and required action (see below and page 23)^{[4][5]}.

S1.	In the past few weeks, have you wished you were dead, or wished you could go to sleep and not wake up?	YES / NO
S2.	In the past few weeks, have you felt that you (or your family) would be better off if you were not alive?	YES / NO
S3.	In the past week, have you had any thoughts about ending your own life?	YES / NO
S4.	Have you ever tried to end your own life? <i>(If yes, ask privately: how, and when.)</i>	YES / NO
S5.	<i>(Ask only if YES to S1-S4)</i> Are you having thoughts of ending your life right now?	YES / NO
S6.	Have you ever started to do anything, or gotten ready to do anything, to end your life -- for example, collecting pills, working out a plan, or writing a note? <i>(If yes, ask: was this within the past 3 months?)</i>	YES / NO

Branching: Always ask S1 and S2. If YES to either, ask S3, S4, and S5. If NO to both, skip directly to S6.^[4]

Suicide Risk Decision Tree

Category	Trigger	Required action
Negative screen	NO to S1-S4 and NO/not-recent to S6	No further action needed today from this screen alone. Continue routine support and monitoring.
Non-acute positive	YES to any of S1-S4 (but NO to S5), or YES to S6 more than 3 months ago	Positive screen. The student cannot leave until a mental health professional completes a brief safety assessment. Do not leave the student alone. ^[4]
Acute / imminent positive	YES to S5, or YES to S6 within the past 3 months, or a recent (past-month) plan or intent	Highest concern. Treat as a medical emergency: continuous supervision, remove access to means, notify the crisis team immediately, and consider emergency services. ^[5]

Any Non-acute or Acute/imminent result moves directly to the Immediate Response Protocol on page 23. Do not continue with the rest of the questionnaire first.^[5]

SECTION 4 — PHYSICAL AGGRESSIVE BEHAVIOR

How often does each statement describe you?

Reactive aggression — lashing out in response to feeling provoked, angry, or threatened.

17. When someone makes me really angry, I yell or shout at them.	0 1 2
18. I hit or push someone back if they hit or push me first.	0 1 2
19. I lose my temper and break or throw things when I'm upset.	0 1 2
20. I get into physical fights when I feel disrespected.	0 1 2
21. I say or do things to hurt someone back when I'm angry, even if I don't mean to.	0 1 2
22. Small annoyances can make me want to physically lash out.	0 1 2

Proactive aggression — planned, goal-directed aggression, not necessarily provoked.

23. I have hurt someone on purpose to get something I wanted.	0 1 2
24. I have threatened or pushed someone around just to show I'm in charge.	0 1 2
25. I have planned ahead of time to hurt or scare someone.	0 1 2
26. I have joined in hurting someone even though they hadn't done anything to me.	0 1 2
27. I have used physical force to make someone do what I wanted.	0 1 2
28. I have carried something (like a weapon or object) planning to use it against someone.	0 1 2

Scale: 0 = Never · 1 = Sometimes · 2 = Often. Elevated proactive-aggression scores combined with high Section 5 scores warrant closer attention than reactive aggression alone.^[6]

SECTION 5 — VIOLENCE ATTITUDES

How much do you agree with each statement?

These items ask about attitudes toward violence in general, not specific incidents. Some items are reverse-scored (marked below) -- a high score on a reverse-scored item reflects a low endorsement of violence.

29. Watching or hearing about a fight is exciting to me.	0 1 2 3 4
30. I enjoy movies, games, or videos with a lot of violence more than other kinds.	0 1 2 3 4
31. People who avoid fights are weak or cowardly.	0 1 2 3 4
32. Handling a weapon (real or toy) makes me feel powerful.	0 1 2 3 4
33. I admire people who solve their problems with force.	0 1 2 3 4
34. It's okay to hurt someone if they deserve it.	0 1 2 3 4
35. I don't need to fight, because there are better ways to solve problems. (reverse-scored)	0 1 2 3 4
36. Seeing someone get hurt bothers me, even if I don't know them. (reverse-scored)	0 1 2 3 4
37. I'd rather walk away from a conflict than get physical, even if others think less of me for it. (reverse-scored)	0 1 2 3 4
38. Thinking about hurting someone else almost never crosses my mind. (reverse-scored)	0 1 2 3 4

Scale: 1 = Strongly disagree · 2 = Disagree · 3 = Neutral · 4 = Agree · 5 = Strongly agree

SECTION 6 — STRESS & PRESSURE

Academic stress and general life stress

This section is descriptive, not diagnostic -- there is no emergency action tied to any score. It measures two related but distinct things: pressure connected specifically to **school and academic performance**, and **general life stress** not tied to any one source. Score and interpret the two subscales separately.

Academic stress — “In the past 30 days, how often have you felt this way about school?”

39. I felt like I had too much homework or schoolwork to finish in the time I had.	0 1 2 3 4
40. I felt nervous or on edge before or during a test or exam.	0 1 2 3 4
41. I worried about not getting the grades I wanted.	0 1 2 3 4
42. I felt pressure from my parents or family to perform better in school.	0 1 2 3 4
43. I felt pressure from my teachers or school to perform better.	0 1 2 3 4
44. I felt like I wasn't as good as other students, or that I was falling behind.	0 1 2 3 4
45. I had trouble sleeping, eating, or relaxing because I was worried about schoolwork.	0 1 2 3 4

Scale: 0 = Never · 1 = Once or twice · 2 = 2-3x a month · 3 = About once a week · 4 = Several times a week or more

General life stress — “Over the past week, how much did this apply to you?” This subscale reproduces the Stress subscale of the DASS-21^[20], a widely used and freely reusable measure, with its validated item wording^[21].

46. I found it hard to wind down.	0	1	2	3
47. I tended to over-react to situations.	0	1	2	3
48. I felt that I was using a lot of nervous energy.	0	1	2	3
49. I found myself getting agitated.	0	1	2	3
50. I found it difficult to relax.	0	1	2	3
51. I was intolerant of anything that kept me from getting on with what I was doing.	0	1	2	3
52. I felt that I was rather touchy.	0	1	2	3

0**1****2****3**

Did not apply to me at all

Applied to me to some degree, or
some of the timeApplied to me to a considerable
degree, or a good part of timeApplied to me very much, or most of
the time

Scoring note: sum the 7 raw responses (0-3 each, range 0-21), then multiply the total by 2 (final range 0-42) for comparability with published DASS-21 norms^[22].

This subscale was validated from adolescence upward. For ages 6-9, self-report of internal states such as “winding down” or feeling “agitated” is unreliable -- for this age band, rely on the academic-stress subscale plus informal caregiver or teacher observation instead of administering the general life-stress items^[20].

SECTION 7 — MEDIA EXPOSURE

Social media use and TV/movie viewing habits — descriptive only, not scored

This section is **descriptive only** -- it does not produce a score, band, or risk level. It builds a snapshot of how much time a respondent spends on social media and watching TV or movies, which platforms and content types they gravitate to, and how often they run into violent or targeting content^{[23][24]}. Read the results alongside Section 4 (Physical Aggression) and Section 5 (Violence Attitudes) -- see the cross-domain flag in the Scoring & Interpretation Guide.

Screen time

53. On an average day, about how much time do you spend on social media (for example Instagram, YouTube, TikTok, Snapchat, WhatsApp, or X)?	0 1 2 3 4
54. On an average day, about how much time do you spend watching TV shows, movies, or streaming video (not counting social media)?	0 1 2 3 4

Scale: 0 = Less than 1 hour · 1 = 1-2 hours · 2 = 2-4 hours · 3 = 4-6 hours · 4 = More than 6 hours

Social media -- platforms & content

55. Which of these do you use regularly? (check all that apply) ☐ Instagram ☐ YouTube ☐ TikTok ☐ Snapchat ☐ WhatsApp ☐ Facebook ☐ X (Twitter) ☐ Discord ☐ YouTube Shorts / Reels ☐ None of these ☐ Other	
56. How often do you come across violent or graphic content on social media (real fights, blood, or people getting badly hurt)?	0 1 2 3 4
57. How often do you see posts, comments, or messages that gang up on, threaten, or make fun of one specific person online?	0 1 2 3 4
Scale: 0 = Never · 1 = Rarely · 2 = Sometimes · 3 = Often · 4 = Very often	
58.	Do you follow any accounts or pages that are mostly about fighting, weapons, gangs, or crime? YES / NO

TV & movies -- type of content**59.** Which of these do you watch most often? (check all that apply)

☐ Action / superhero
 ☐ Horror / thriller
 ☐ Crime / gangster
 ☐ Comedy
 ☐ Family / animation
 ☐ Drama / romance
 ☐ Sports
 ☐ Reality TV
 ☐ Documentary
 ☐ Other

60. How often do the shows or movies you watch most include fighting, shooting, or other physical violence?**0 1 2 3 4**

Scale: 0 = Never · 1 = Rarely · 2 = Sometimes · 3 = Often · 4 = Very often

61.

Have you ever watched a movie or show rated for adults (18+ / R-rated / A-rated) that shows graphic violence?

YES / NO / NOT SURE

No score is calculated for Section 7. Use the screen-time bands, checklists, and frequency answers above to describe a respondent's media diet in a follow-up conversation -- see the Scoring & Interpretation Guide for how to read this section alongside Sections 4 and 5.

Scoring & Interpretation Guide

Score Sections 1, 2, 4, 5, and 6 by summing item responses per subscale. Section 3 is never summed -- use the decision tree on page 11 instead. Section 6 bands are purely descriptive -- unlike Section 3, no band in Section 6 triggers a mandatory action; use professional judgment to decide on follow-up. Section 7 is never summed either -- it produces a descriptive media-exposure profile, not a score or band.

Section 1 — Bullying: Victimization (8 items, range 0-32)

Score	Level	Meaning
0-6	Minimal	Little to no reported victimization in the past 30 days.
7-15	Occasional	Some victimization -- monitor and check in periodically.
16-23	Frequent	Regular victimization -- counselor follow-up recommended.
24-32	Severe	Frequent, intense victimization -- immediate intervention recommended.

Section 2 — Bullying: Perpetration (8 items, range 0-32)

Score	Level	Meaning
0-6	Minimal	Little to no reported bullying behavior.
7-15	Occasional	Some bullying behavior -- early behavioral intervention recommended.
16-23	Frequent	Regular bullying behavior -- structured behavioral intervention recommended.
24-32	Severe	Frequent, intense bullying behavior -- immediate intervention and safety planning recommended.

Section 4 — Physical Aggression

Reactive subscale (6 items, range 0-12)

Score	Level	Meaning
0-2	Minimal	Little reactive aggression reported.
3-6	Moderate	Some reactive aggression -- monitor triggers and coping skills.
7-9	Elevated	Frequent reactive aggression -- recommend anger-management support.
10-12	High	Very frequent reactive aggression -- recommend prompt intervention.

Proactive subscale (6 items, range 0-12)

Score	Level	Meaning
0-1	Minimal	Little planned/instrumental aggression reported.
2-4	Moderate	Some proactive aggression -- monitor and follow up.
5-7	Elevated	Notable proactive aggression -- recommend behavioral intervention.
8-12	High	Frequent planned aggression -- recommend immediate intervention; research links this pattern to more severe outcomes than reactive aggression alone. ^[6]

Section 5 — Violence Attitudes (10 items, range 10-50)

Reverse-score items 35-38 (score = 6 minus the raw response) before summing all 10 items.

Score	Level	Meaning
10-19	Low	Little endorsement of violence as acceptable or exciting.
20-29	Moderate	Some endorsement -- typical range for many adolescents; monitor in context.
30-39	Elevated	Above-average endorsement of violence -- recommend follow-up conversation.
40-50	High	Strong endorsement of violence -- recommend clinical follow-up, especially alongside elevated Section 4 proactive-aggression scores.

Cross-domain flag: a student who is High on both Section 4 (proactive) and Section 5 (violence attitudes) represents a distinct pattern -- planned aggression paired with a positive view of violence -- and should be prioritized for professional follow-up regardless of any single band cutoff.^{[6][8]}

Section 6 — Stress & Pressure

These bands are descriptive only. There is no mandatory-action trigger in Section 6 -- use them to guide a conversation, not to force a referral.

Academic stress subscale (7 items, range 0-28)

Score	Level	Meaning
0-6	Low	Little reported academic stress in the past 30 days.
7-14	Moderate	Some academic stress -- a typical range for many students; monitor in context.
15-21	Elevated	Above-average academic stress -- consider a supportive check-in about workload and expectations.
22-28	High	High academic stress -- recommend a follow-up conversation with a counselor about workload, expectations, or study support.

General life-stress subscale (DASS-21 Stress items, range 0-42 after doubling)

Use the youth cutoffs (DASS-Y) for respondents under 18, and the adult cutoffs for ages 18-50^[22].

Youth cutoffs (ages 6-17)

Score	Level	Meaning
0-11	Normal	Stress in the normative range for youth.
12-13	Mild	Mildly elevated stress relative to youth norms.
14-16	Moderate	Moderately elevated stress relative to youth norms.
17-18	Severe	Severely elevated stress relative to youth norms.
19+	Extremely Severe	Extremely elevated stress relative to youth norms.

Adult cutoffs (ages 18-50)

Score	Level	Meaning
0-14	Normal	Stress in the normative range for adults.
15-18	Mild	Mildly elevated stress relative to adult norms.
19-25	Moderate	Moderately elevated stress relative to adult norms.
26-33	Severe	Severely elevated stress relative to adult norms.
34+	Extremely Severe	Extremely elevated stress relative to adult norms.

These severity labels come directly from the published DASS-21/DASS-Y norms^[22]; they describe how a respondent's stress level compares to normative samples and do not, by themselves, indicate a clinical diagnosis or require a specific action.

Section 7 — Media Exposure (descriptive only, no score)

This section does not produce a score or a risk band. It is a snapshot of how much time a respondent spends on social media and watching TV or movies, which platforms and content types they gravitate to, and how often they encounter violent, aggressive, or targeting content. Read it descriptively -- use the screen-time bands, checklists, and frequency answers to build a picture of a student's media diet, not to calculate a total.

Cross-domain flag: a respondent who reports frequent exposure to violent content (Often or Very often on item 56 or 60, or a Yes on item 58 or 61) alongside an Elevated or High score on Section 4 (proactive aggression) and/or Section 5 (violence attitudes) shows a pattern documented in longitudinal research linking heavy media-violence exposure to aggressive behavior and pro-violence attitudes^{[23][24][25]}. As with the Section 4/5 flag, this is not a diagnostic threshold on its own -- treat it as additional context that raises the priority of a follow-up conversation, not a stand-alone trigger for action.

Immediate Response Protocol

Follow these steps the moment Section 3 returns a Non-acute or Acute/imminent positive screen. Do this before scoring any other section.

1. Stay calm and stay with the student. Never leave them alone, even briefly.
2. If the result is **Acute/imminent**: treat it as a medical emergency. Contact your school's crisis response coordinator immediately, call for emergency medical assistance if needed, and remove access to any means of self-harm.^[5]
3. If the result is **Non-acute positive**: the student cannot leave until a mental health professional has completed a brief safety assessment.^[4]
4. Contact the designated on-site mental health professional or counselor right away -- do not wait until the end of the school day.
5. Notify the parent/guardian as soon as possible, following your school's protocol, unless doing so would place the student at further risk (for example, suspected abuse at home) -- in that case, follow your child-protection reporting procedures instead.^[11]
6. Document every step: time, who was contacted, what was decided, and next steps.
7. Develop a brief safety plan with the student and a caregiver before the student leaves the building.
8. Arrange a specific, time-bound follow-up (for example, a check-in within 24-48 hours) and a re-entry and support plan if the student is referred out or hospitalized.
9. Keep all records confidential and share them only with those directly responsible for the student's safety.

Crisis Resources

Print this page and keep it visible at every screening station.

India

Service	Contact	Notes
Tele-MANAS (National Tele Mental Health Programme)	14416 or 1800-89-14416	24x7, multilingual, Ministry of Health & Family Welfare
KIRAN Mental Health Rehabilitation Helpline	1800-599-0019 (toll-free)	24x7, 13 languages, Ministry of Social Justice & Empowerment
iCall Psychosocial Helpline	9152987821	Mon-Sat, 10:00-20:00 -- Tata Institute of Social Sciences
Vandrevala Foundation	+91 9999 666 555 (call/WhatsApp)	24x7x365, free counseling
AASRA	022-27546669	24-hour helpline, Navi Mumbai

International

Service	Contact	Notes
988 Suicide & Crisis Lifeline (United States)	Call or text 988	24x7, free and confidential

Sources: Tele-MANAS and KIRAN details from Government of India releases^{[12][13]}; iCall^[14]; Vandrevala Foundation^[15]; AASRA^[16]; 988 Lifeline^[17]. Verify current numbers before printing, as helpline details can change.

Documentation & Follow-Up Log

Complete this log for every screening session where any section returns a Frequent/Severe, Elevated/High, or any positive Section 3 result. Keep completed logs confidential and secured.

Field	Entry
Date & time	
Student initials / ID	
Sections administered	
Section 3 result (Negative / Non-acute / Acute)	
Other flagged sections & levels	
Action taken	
Professional's name & signature	
Parent/guardian notified? (Y/N, time)	
Follow-up date scheduled	

Sample parent/guardian notification note

“We are reaching out because, during a routine wellbeing check-in today, [student] shared some things that we take seriously and want to make sure [student] gets the right support for. We have connected [student] with [name/role of mental health professional], and we would like to set up a time to talk with you as soon as possible about next steps. [Student] is safe and was not left alone.” Adapt this note to your school's protocol and the specific situation before using it.

References & Sources

This instrument is original; the sources below are the publicly documented frameworks and guidance that informed its design and the crisis-resource listings. Item text from copyrighted instruments (marked in the source literature) was not reproduced verbatim.

1. Solberg, M. E., & Olweus, D. (2003). Olweus Bully/Victim Questionnaire, as documented in the CDC bullying-measurement compendium. <https://ed.buffalo.edu/content/dam/ed/alberti/docs/Measuring-Bullying-Victimization.pdf>
2. Espelage, D. L., & Holt, M. K. (2001). Illinois Bully Scale, as documented in the CDC bullying-measurement compendium. <https://ed.buffalo.edu/content/dam/ed/alberti/docs/Measuring-Bullying-Victimization.pdf>
3. Green, J. G. et al. (2018). California Bully Victimization Scale-Retrospective (CBVS-R) instrument. https://www.bu.edu/schoolmentalhealth/files/2021/04/CBVS-R_VictimizationScale.pdf
4. National Institute of Mental Health. Ask Suicide-Screening Questions (ASQ) Toolkit. https://www.nimh.nih.gov/sites/default/files/documents/research/research-conducted-at-nimh/asq-toolkit-materials/asq-tool/screening_tool_asq_nimh_toolkit.pdf
5. Posner, K. et al. Columbia-Suicide Severity Rating Scale (C-SSRS), Screen Version. <https://www.cms.gov/files/document/cssrs-screen-version-instrument.pdf>
6. Raine, A. et al. (2006). Reactive-Proactive Aggression Questionnaire (RPQ). <https://bpb-us-w2.wpmucdn.com/web.sas.upenn.edu/dist/1/681/files/2022/04/Reactive-Proactive-Aggression-Questionnaire-RPQ.docx>
7. Buss, A. H., & Perry, M. (1992). The Aggression Questionnaire. *Journal of Personality and Social Psychology*, 63(3), 452-459. <https://citeseerx.ist.psu.edu/document?repid=rep1&type=pdf&doi=1ad0757338faf541e76a25645a571ae010abd4ad>
8. Huesmann, L. R., Guerra, N. G., Miller, L., & Zelli, A. (1989/2019). Normative Beliefs about Aggression Scale (NOBAGS) manual. https://arg.isr.umich.edu/wp-content/uploads/2022/12/NOBAGS.MEM_.rev2019.pdf
9. CDC. Measuring Violence-Related Attitudes, Behaviors, and Influences Among Youths (2nd ed., 2005). https://www.wcasa.org/wp-content/uploads/2020/05/Evaluation_11.-CDC_Measuring-Violence-Related-Attitudes-Behaviors-and-Influences-Among-Youths.pdf
10. Columbia Lighthouse Project. C-SSRS Frequently Asked Questions (age ranges, safety of asking). <https://cssrs.columbia.edu/the-columbia-scale-c-ssrs/faq/>
11. SAMHSA. Preventing Suicide: A Toolkit for High Schools (HHS Publication No. SMA-12-4669, 2012). https://sptsusa.org/wp-content/uploads/2015/05/SAMHSA_HS_Suicide_Prevention_Toolkit.pdf
12. Government of India, Directorate General of Health Services. National Mental Health Programme / Tele-MANAS. <https://dghs.mohfw.gov.in/national-mental-health-programme.php>
13. Press Information Bureau, Government of India. 24x7 Toll-Free Mental Health Rehabilitation Helpline KIRAN. <https://www.pib.gov.in/pressreleaseshare.aspx?prid=1652240>
14. iCALL Psychosocial Helpline, Tata Institute of Social Sciences. <https://icallhelpline.org/>
15. Vandrevalla Foundation. Free Counseling / Contact Us. <https://www.vandrevallafoundation.com/free-counseling/contact-us>
16. AASRA. Contact page. <https://www.aasra.info/contact.html>
17. 988 Suicide & Crisis Lifeline. About page. <https://988lifeline.org/about/>
18. Sun, J., Dunne, M. P., Hou, X., & Xu, A. (2011). Educational Stress Scale for Adolescents (ESSA): development, validity, and reliability, as documented in subsequent validation literature. <https://pmc.ncbi.nlm.nih.gov/articles/PMC12622241/>
19. Adolescence Stress Scale (ADOSS), India-validated against salivary cortisol, ages 10-17. <https://pmc.ncbi.nlm.nih.gov/articles/PMC10159577/>
20. Lovibond, S. H., & Lovibond, P. F. (1995). Manual for the Depression Anxiety Stress Scales (2nd ed.). Sydney: Psychology Foundation of Australia -- official DASS site confirming public-domain status. <http://www2.psy.unsw.edu.au/groups/dass/>
21. DASS-21 full item wording and administration format. <https://maic.qld.gov.au/wp-content/uploads/2016/07/DASS-21.pdf>
22. DASS-21 adult severity cutoffs (Bristol University scoring guide) and DASS-Y youth severity cutoffs, derived by Szabo & Lovibond (2022) using the same percentile ranges as the adult DASS. <https://www.bristol.ac.uk/media-library/sites/sps/documents/c-change/dass-twenty-one-scoring-and-interpretation.pdf>

23. den Hamer, A. H., Konijn, E. A., Plaisier, X. S., Keijer, M. G., Krabbendam, L., & Bushman, B. J. (2017). The Content-based Media Exposure Scale (C-ME): Development and validation. *Computers in Human Behavior*, 72. https://research.vu.nl/ws/files/273736057/The_Content_based_Media_Exposure_Scale_C_ME_.pdf
24. Gentile, D. A. et al. (2004). Exposure to Violent Media Questionnaire (ETVMQ) structure and scoring, as documented in subsequent replication and validation literature. <https://pmc.ncbi.nlm.nih.gov/articles/PMC11747719/>
25. Huesmann, L. R. et al. (2003). Longitudinal relations between children's exposure to TV violence and their aggressive and violent behavior in young adulthood. *Developmental Psychology*, 39(2). <https://www.apa.org/pubs/journals/releases/dev-392201.pdf>
26. HBSC (Health Behaviour in School-aged Children) international survey. Screen-time measurement protocol and reliability of screen-time items among adolescents. <https://pmc.ncbi.nlm.nih.gov/articles/PMC12829222/>

Disclaimer: This instrument is an original work and is not affiliated with, endorsed by, or reproduced from any of the copyrighted instruments referenced above. It is intended solely as a screening aid for trained professionals and does not replace a full clinical evaluation, diagnosis, or treatment plan.